

SECTION 17

Federal Recognition, Provincial Clawback

The pattern of provincial disability policy operating against federal disability recognition, and the vaccine-injured Albertans caught in the pattern

Every time the federal government recognizes disability and attaches supports to that recognition, Alberta claws back, strips, or administratively neutralizes those supports. This is not accidental. It is policy.

This section documents a structural pattern in Alberta provincial disability policy: the consistent provincial response to federal recognition of disability is not to amplify that recognition, but to offset, claw back, or administratively minimize it.

The section presents four instances of the pattern, ending with the case that unifies them — the federal Vaccine Injury Support Program, its approved claimants, and the structural conflict of interest created by Alberta's administration of disability benefits for a population whose injuries were caused, in part, by provincial vaccine mandates.

The purpose of the section is not to relitigate the epidemiology of COVID-19 vaccination. That is not the question before this report. The question before this report is: when the federal government has formally recognized a population of disabled people — through the Disability Tax Credit, the Registered Disability Savings Plan, the Canada Disability Benefit, or the Vaccine Injury Support Program — what does the provincial administration of that population's supports look like?

The answer, documented in the sections that follow, is: it looks like clawback, deduction, administrative burden, and in some cases, non-acknowledgement of the federal determination itself.

Instance 1: the Disability Tax Credit

The federal Disability Tax Credit (DTC) is a non-refundable tax credit available to Canadians with prolonged and severe physical or mental impairments. It is administered by the Canada Revenue Agency. Eligibility is established through a medical certification (Form T2201, Part B) completed by a physician or authorized health professional.

The DTC functions, in the federal framework, as the gateway to additional federal disability supports — including the Registered Disability Savings Plan and the Canada Disability Benefit. Eligibility for the DTC is, in effect, federal recognition of the individual as a person with a qualifying disability.

According to the Government of Canada's 2024 Disability Advisory Committee report, only approximately 24 percent of individuals likely eligible for the DTC have completed a successful application. The Committee's own finding is that the application process is time-consuming and complex, and that "more support and accessibility in the process is needed for applicants to successfully complete the process."

Two specific structural barriers affect Alberta applicants:

- Medical forms to certify the DTC cost between \$300 and \$400 per application when completed by a private health practitioner, as documented in CBC News reporting on Alberta AISH recipients during 2025. For an AISH recipient living on \$1,901 per month, this is a meaningful financial barrier.
- Only licensed health professionals can complete Part B of Form T2201. In Alberta, more than 650,000 residents do not have a family doctor. For an applicant without a regular physician, completing the DTC application may require a dedicated appointment with a walk-in physician willing to fill out the form — a service that is not guaranteed, universal, or cost-exempt.
- The DTC process itself takes time. CBC coverage in July 2025 documented Alberta physicians reporting queues for DTC form completion with realistic timelines extending to September 2026 — a full year or more beyond the provincial deadline set for AISH recipients to communicate their CDB application status.

The federal government committed, in Budget 2024, to cover the cost of the medical forms. As of the drafting of this report in April 2026, the reimbursement model has not been finalized. AISH may advance the cost on a repayable basis, but the program's own guidance notes that "details of this reimbursement model have not yet been announced."

The result is that federal disability recognition, for Alberta residents, carries an administrative and financial barrier that compounds the very condition it is designed to recognize. The DTC exists. Many disabled Albertans who qualify do not successfully apply. The gap between eligibility and enrollment is not neutral. It is itself a form of administrative filtering.

Instance 2: the Registered Disability Savings Plan

The Registered Disability Savings Plan (RDSP) is a federally-structured long-term savings vehicle for Canadians with qualifying disabilities. It was introduced by the federal government in 2008 specifically to support the financial security of disabled individuals across their lifetime.

Two federal grant and bond streams are attached to the RDSP:

- The Canada Disability Savings Grant (CDSG): up to \$3,500 in annual matching contributions from the federal government, to a lifetime maximum of \$70,000.

- The Canada Disability Savings Bond (CDSB): up to \$1,000 annually for low-income beneficiaries, to a lifetime maximum of \$20,000 — available without any personal contribution requirement.

The RDSP and its attached grants are a federal determination that individuals with permanent disabilities require specific lifetime financial support — distinct from, and additional to, provincial disability assistance. The federal government has allocated these grants to this population on the explicit basis that permanent disability creates financial burden that will persist across the lifespan.

Alberta's AISH program fully exempts the RDSP. Under both AISH eligibility rules and the announced rules for the Alberta Disability Assistance Program (ADAP) that takes effect July 1, 2026, RDSP assets do not count toward the \$100,000 non-exempt asset limit, and RDSP income (including federal grants, bonds, rollovers, and investment earnings) is excluded from income calculations. This is confirmed by the Government of Alberta's AISH eligibility documentation, by the Alberta legal community, and by the 2026 Registered Disability Savings Plan Reference Guide published by ATB Financial, which states plainly: "assets and income from an RDSP will not affect your AISH or ADAP eligibility."

The RDSP is therefore the one federal disability recognition mechanism Alberta does not claw back or administratively minimize. That is significant — not as evidence of general provincial generosity, but as the exception that clarifies the pattern. If Alberta's consistent policy were to exempt federal disability instruments as a matter of fiscal principle, the RDSP exemption would not be notable. Because the RDSP is the only exempt instrument among those examined in this section — and because the Canada Disability Benefit, which serves a similar population under similar federal authority, is not exempt — the provincial position reveals itself to be selective, not principled.

The operative distinction, as best the pattern can be reconstructed from provincial documents, appears to be structural. The RDSP is a long-term savings vehicle — its federal contributions are not paid out as monthly income that would otherwise compete with AISH's monthly benefit amount. The CDB, by contrast, is a monthly federal cash benefit that directly overlaps with the AISH monthly benefit the province administers. Alberta exempts the federal mechanism that does not displace its own visible payment role; it claws back the federal mechanism that would, without clawback, expose the relative inadequacy of provincial supplementation. That is a fiscal and political calculation, not a principled disability policy.

Instance 3: the Canada Disability Benefit — the unambiguous case

The Canada Disability Benefit (CDB) was introduced by the federal government in 2024, with payments beginning in June 2025. The benefit provides up to \$2,400 per year (\$200 per month) to adult Canadians who hold a valid Disability Tax Credit certificate and meet the income eligibility threshold.

The federal government's stated purpose is explicit. As noted in the federal enabling legislation and accompanying policy documents, the CDB is intended as a supplement to provincial and territorial disability supports, designed to lift Canadians with disabilities out of poverty. The federal government specifically called on provinces and territories to exempt CDB payments from counting as income in relation to provincial or territorial supports.

Every other province and territory in Canada has done so. Alberta is the only province or territory that has not.

Alberta is the only province or territory in Canada that has announced it will claw back the Canada Disability Benefit from AISH recipients. Every other province and territory exempts it.

The Alberta policy, announced in March 2025 and confirmed through a letter sent to AISH recipients on July 2, 2025, requires AISH clients to apply for the Canada Disability Benefit. Recipients who receive the CDB have the \$200 amount deducted dollar-for-dollar from their AISH monthly benefit. Clients who have not communicated their CDB application status to AISH by September 5, 2025 also see the \$200 reduction applied beginning in the October 2025 period of assistance.

The net effect: AISH recipients in Alberta who receive the CDB do not see any increase in their monthly disability income. The federal dollar is received, the equivalent provincial dollar is withdrawn, and the recipient's total monthly disability benefit is unchanged.

This is not an accidental outcome. It is the explicit policy of the Government of Alberta. It is also, according to the National Disability Network — representing more than 40 disability organizations — “contrary to the UN Convention on the Rights of Persons with Disabilities and UN Declaration on the Rights of Indigenous Peoples, to which Canada is a signatory.”

The policy is numerically linked to other provincial measures. The 2025 Alberta budget reduced AISH program funding by approximately \$49 million from the third-quarter forecast of the previous year. Inclusion Alberta and The Tyee have both documented that this \$49 million reduction was made in anticipation of the federal CDB launch — that is, the province reduced its own disability spending by approximately the amount the federal government was about to add. The same \$49 million appears elsewhere in this report series in the context of the MHCare Medical procurement irregularities, where it has a separate documentary trail. The numerical coincidence between the AISH reduction and the MHCare corruption loss, and between both and the arrival of federal disability supplementation, is documented and verifiable.

Trish Bowman, CEO of Inclusion Alberta, characterized the provincial policy in May 2025: “At a time when the province is cutting income tax because it made an election promise to do so, they're basically imposing a 10-per-cent tax on disability.”

Tony Flores, Alberta's first advocate for persons with disabilities, addressed the broader impact in November 2025: "Can we live on \$1,700 a month with the increasing cost of living? It's unrealistic."

Gregory McMeekin, Alberta's current Advocate for Persons with Disabilities — a position appointed by the provincial government — stated the structural reality concisely: "Once we report, it's up to the province to decide what they want to do with the information." The statement is consistent with the placation architecture documented in Section 16: the instruments of recognition exist, the reports are filed, the accountability structures are staffed, and the substantive change does not follow.

The administrative trap: how the clawback was operationalized

The documentary record of how the CDB clawback was actually implemented is an important part of this instance, because it demonstrates the placation architecture operating in real time — not as rhetorical pattern, but as specific administrative failure with specific human cost.

The Canada Disability Benefit is gated by the federal Disability Tax Credit. Without a valid DTC certificate, an applicant cannot receive the CDB. This is federal design, not provincial. However, the practical consequence for AISH recipients is this: the DTC is a non-refundable tax credit. It reduces taxable income. For most AISH recipients, whose AISH benefits are non-taxable and whose other income is minimal, the DTC historically provided no direct financial benefit. There was no reason — financial, administrative, or otherwise — for the overwhelming majority of AISH recipients to have pursued DTC certification before the CDB existed. Many had not done so.

In 2024, the federal government announced the CDB with DTC eligibility as a mandatory upstream requirement. In March 2025, Alberta announced the CDB clawback. In July 2025, Alberta sent AISH recipients a letter directing them to apply for the CDB, with a September 5, 2025 deadline to notify AISH of their application status, and October 2025 as the date the \$200 clawback would begin if they did not.

The letter did not adequately communicate the DTC prerequisite. AISH recipients across the province understood themselves to be applying for the CDB directly. Many did so, only to be denied because they had no DTC certificate. The denials were, in each case, correctly issued at the federal level. The administrative failure was provincial — in the original letter to recipients, which directed them toward an application they were not yet eligible to submit, and in the absence of clear instruction that the DTC must be obtained first.

AISH contact centre staff did not consistently have accurate information about the DTC prerequisite either. Recipients calling for guidance, advocacy organizations attempting to refer members, and even some health professionals were receiving inconsistent or incomplete information through official provincial channels during the period the deadline was approaching.

The parallel reality of the DTC application itself compounded the trap. Form T2201 Part B, the medical certification, is completed by a physician or nurse practitioner. Fees for completion in Alberta during 2025 ranged from \$300 to \$400. Ivy Hays, a Coronation, Alberta AISH recipient of approximately twenty years' standing with a cardiac disability profile, reported to CBC News in July 2025 that her physician charged \$400 for the form and refused instalment payments. Hays reported, of learning the amount required: "My heart hit the floor. I have no living family. I have no way to come up with that \$400."

Physicians tasked with completing the form reported queues that put the realistic timeline far beyond the provincial deadline. Dr. Kara Wilson, cited in the CBC coverage, estimated completion times of two to three hours per form and reported that her existing queue — for the DTC forms alone — would not realistically be cleared until September 2026. That is a full year past the original September 2025 deadline, and approximately eighteen months past the October 2025 clawback commencement. Dr. Ginetta Salvalaggio, an Edmonton family physician, told CBC: "Why target the disabled community in particular in the name of saving a buck?", noting that the province ended its fiscal year with an \$8.3 billion surplus.

In the face of widespread panic and documented provider overwhelm, Minister Jason Nixon held a press conference in late July 2025 at which he clarified that AISH recipients were not expected to complete the federal application process by September 5 — only to notify AISH of the status of their application by that date. The deadline was subsequently extended to February 28, 2026, and the clawback commencement pushed to April 2026.

The extension is itself the tell. A deadline that is extended repeatedly by the government that set it is a deadline that was unrealistic when set. The original September 5 deadline existed in a documentary form — a letter sent to every AISH recipient in Alberta — while the administrative reality required to meet it did not exist at the provincial level. The extension then required a second round of official communications, advocacy-organization clarifications, and recipient adjustments. Each round imposed additional cognitive and logistical burden on a population that, by definition, has limited capacity for exactly that kind of administrative navigation.

The structural function of the trap is consistent with the broader pattern of this report. A program exists that, on its face, responds to disability poverty — the federal CDB. A provincial policy exists that, on its face, requires recipients to access available federal supports — the CDB application requirement. The instruments are in place. The mechanisms are visible. The accountability structures — the Minister's office, the AISH contact centre, the advocacy partnerships — are staffed. And the operational reality, in the months during which the policy was first applied, was mass denial of federally-eligible applicants because the province had not communicated the upstream requirement to the people it was directing to apply.

It is possible, on a narrow reading, to describe this as administrative oversight. It is more accurate, on a broader reading, to describe it as the placation architecture of Section 16 operating in a specific instance: the recognition exists, the support has been announced, the

process appears responsive, and the operational reality produces the opposite outcome — denial, delay, panic, and the quiet continuation of the clawback.

The advocacy community's response is itself informative. Plan Institute, Inclusion Alberta, Disability Alliance Canada, and the Canadian Down Syndrome Society all built parallel support infrastructure during the second half of 2025 specifically to guide AISH recipients through the DTC–CDB sequence that the provincial communications had failed to clarify. These are not the organizations that were supposed to be doing this work. Alberta's \$3.6 billion disability budget, repeatedly cited by provincial officials as evidence of generosity, includes no allocation equivalent to the navigation services the advocacy community assumed at its own cost. The work was done. It was done by the wrong party.

The coordinated removal of navigation supports

The DTC–CDB administrative trap did not happen in isolation. It happened while the Government of Alberta was simultaneously dismantling the two support structures that would have allowed affected recipients to navigate the process: individual caseworker relationships and community advocacy organizations. Documenting these three actions as coordinated rather than accidental requires only the existing public record.

First — individual caseworkers. For most of AISH's forty-seven-year history, recipients were assigned an individual caseworker: a specific named person with access to their complete file, their medical history, their benefit history, and their personal circumstances. Beginning in 2024 and accelerating through 2025, the Government of Alberta restructured this model province-wide. Individual caseworker assignments were eliminated and replaced by a centralized contact centre. The Calgary region transitioned September 18, 2025. Direct worker phone lines across the remainder of the province were scheduled to be shut off by November 17, 2025.

Don Slater, disability advocate and AISH recipient, summarized the operational consequence in Yahoo News Canada, September 2025: "When you had a worker, the buck stopped here. That person would take care of your issue no matter how complicated or simple. With the call centre, sometimes you can get a hold of someone more easily, but if your issue is complicated it has a tendency to get shuffled aside or it gets lost in the paperwork."

Second — the staffing collapse underneath the restructure. The call-centre transition did not occur in a staffing vacuum. It occurred alongside a documented reduction in the workforce supporting it. Sandra Azocar, President of the Alberta Union of Provincial Employees, reported in February 2024: "The government has axed the jobs of dozens of case workers and is refusing to fill vacant positions. People with disabilities are tired of waiting to be assessed and to get support. Some have been waiting for more than two years because the government has instituted a freeze on handling new applications." By January 2025, AUPE reported that members were carrying caseloads of up to 200 files each, against a recommended maximum of 90 to 105. Temporary workers hired during summer 2024 — approximately thirty to forty positions — were subsequently not renewed when a new hiring freeze was imposed.

The arithmetic is straightforward. At the recommended maximum caseload, Alberta's 79,290 AISH recipients require a minimum of 755 to 881 workers operating at capacity. Workers were carrying approximately double the maximum. Vacancies were frozen. Paperwork backlogs ran two to three months behind. Approximately 12,000 families were waiting across AISH, FSCD, and PDD combined. Into this already-collapsing system, the province added the requirement that all 79,000 current AISH recipients be individually reassessed for ADAP placement by July 1, 2026, plus the DTC–CDB compliance requirement described above, plus the ongoing flow of new applications already frozen at the intake stage.

Third — the defunding of advocacy organizations during the precise period the transition was imposing maximum navigation demand. Three advocacy organizations supporting families navigating the Persons with Developmental Disabilities program had their funding ended twelve months early in March 2026 — at the same time the AISH-to-ADAP transition was creating the highest demand for advocacy support in years. The Family Resource Centre of Central Alberta, operated by EPSS, which had served Central Alberta families with disabilities for five years under the Family Managed Services framework, closed April 1, 2026. Its closure was documented by the organization's own public statement and by community response from Betty Smith at the AISH Related Community group on April 6, 2026.

The composite effect of the three actions is a coordinated removal of the scaffolding that would have allowed disabled Albertans to meet the compliance requirements the province was simultaneously imposing. The call-centre restructure eliminated individual continuity of file. The staffing collapse degraded the call-centre's capacity to respond at all. The advocacy-organization defunding removed the community-based navigation support that had historically filled gaps in provincial service. Each action, in isolation, has an official justification. Stacked together, during the exact window when the DTC–CDB–AISH–ADAP compliance architecture was being deployed, the composite effect is the systematic elimination of every support channel through which a disabled recipient could have successfully navigated the process.

The compliance requirements themselves were communicated, in significant part, through the monthly direct-deposit statement — the deposit slip mailed to each AISH recipient before their payment arrives. The AISH caseload includes 28.3 percent of recipients whose primary diagnosis is a cognitive disorder. It includes recipients with acquired brain injuries, autism spectrum disorder, severe depression, and psychotic disorders. The deposit statement, which functions as the primary communication vehicle for compliance requirements carrying financial consequences, is written in standard bureaucratic English with no plain-language alternative, is formatted identically to every other notice with no visual urgency marking, is delivered by mail with no confirmation of receipt or comprehension, and is unaccompanied by any caseworker call or follow-up. It is, in effect, a legal notice with financial consequence delivered to a population defined by its reduced capacity to parse legal notices.

Article 9 of the UN Convention on the Rights of Persons with Disabilities requires that information be provided in accessible formats. Article 21 requires that persons with disabilities

receive information intended for the general public in accessible formats and technologies appropriate to different kinds of disabilities in a timely manner and without additional cost. The Alberta Human Rights Act requires reasonable accommodation. A compliance deadline communicated through bureaucratic shorthand on a deposit statement, to a population 28.3 percent of whom have primary cognitive disorders, after the caseworkers and advocacy organizations that would have helped them parse it have been removed, does not meet these standards. It is not clear that any good-faith interpretation of those obligations would conclude that it does.

The community response to the composite effect is itself a data point. A documented Alberta AISH recipient community group tripled in size over the past year — from approximately 2,000 members to more than 6,000 — according to statements from the group's administrator. Community groups of this kind do not triple in size when recipients feel supported and informed. They triple when recipients feel abandoned and afraid, and when they are actively seeking information and community that the formal system has ceased to provide. The growth of informal recipient-led networks during the same period the formal supports were being removed is not coincidence. It is the predictable consequence of the removal.

Naming this pattern precisely: the Government of Alberta imposed complex compliance requirements on a population defined by its reduced capacity to navigate complex compliance requirements, at the same time that it removed the three primary support structures — individual caseworkers, adequate staffing, and community advocacy organizations — that would have allowed that population to meet those requirements. The province then communicates the requirements through formats inaccessible to the population bound by them. When the population fails to comply, the clawback proceeds. When the population succeeds in complying, the clawback proceeds anyway, because the CDB amount is deducted dollar-for-dollar from AISH regardless.

This is not an administrative failure. It is an administrative design whose outcome is consistent with the fiscal objective the clawback itself serves. The province saves the \$200 per month by clawback where compliance succeeds, and by non-payment where compliance fails. The structural efficiency is clear. The human cost is distributed across a population that, by the province's own medical eligibility determinations, is the population least equipped to bear it.

Instance 4: the Vaccine Injury Support Program — and what it means for Alberta ADAP applicants

The program itself

In December 2020, the Government of Canada announced the Vaccine Injury Support Program (VISP), subsequently launched in June 2021. The program's purpose, in the federal government's own language, was to ensure that people in Canada who experience serious and permanent injury following a Health Canada-authorized vaccine have fair and timely access to

financial support. The program was Canada's first national vaccine injury compensation program, notwithstanding that every other G7 country and the province of Quebec had operated their own programs for years or decades prior.

The program operates on a no-fault basis. Claimants are not required to prove negligence. A medical review board assesses whether the available evidence supports a probable causal link between the vaccine and the injury, and whether the injury is serious and permanent. Approved claims result in compensation — up to \$284,000 for pain and suffering, plus ongoing care and income replacement as applicable.

The federal determination, when a claim is approved, is that the claimant's disability was caused by a Health Canada-authorized vaccine, and that the disability meets the standard of "serious and permanent." That determination is functionally equivalent to AISH's "severe and permanent disability" eligibility threshold. A VISP-approved claimant is, by federal medical-review-board determination, a disabled person.

As of April 1, 2026, the program was renamed the Vaccine Impact Assistance Program (VIAP) and administered directly by the Government of Canada, following concerns about the previous third-party administrator's performance.

The scale

The publicly available program statistics, as reported by the Government of Canada and covered by CBC News and Radio-Canada, are:

- As of December 1, 2025: 3,557 claims submitted.
- 451 claims found inadmissible.
- More than 3,000 claims referred for medical review.
- Over 850 claims still in medical-records collection phase in December 2025.
- Approximately 1,400 claims assessed by the medical review board.
- 252 claims approved.
- Over \$21 million paid out in compensation to approved claimants.
- 328 denied claimants filed appeals; 10 appeals overturned the original decision as of end of 2024.
- Quebec's separate Vaccine Injury Compensation Program received 302 COVID-era claims with 57 approved.

The program's budget trajectory is itself instructive. The original allocation was \$75 million for five years, based on modelling from routine immunization programs. Budget 2024 added \$36.4 million, bringing the total envelope to \$111.4 million. The Public Health Agency of Canada's own Question Period briefing note — a document prepared for Parliamentary questioning —

describes the original budget as “insufficient in response to the largest vaccination campaign in Canadian history with novel vaccines for which the safety profile was not fully understood.”

That sentence is the federal government’s own characterization. It appears in an internal briefing document that became publicly available through Access to Information. It acknowledges that (a) the vaccination campaign was unprecedented in scale, (b) the vaccines were novel, (c) the safety profile was not fully understood at the time of deployment, and (d) the compensation program budget had to be approximately 48 percent larger than originally forecast to respond to actual demand.

The peer-reviewed medical literature

The peer-reviewed medical literature on adverse events following COVID-19 mRNA vaccination is extensive. The pattern most clearly established is myocarditis and pericarditis, particularly in young males after second doses of mRNA vaccines. The published sources include:

- The New England Journal of Medicine (Witberg et al., 2021): Israeli Clalit Health Services database analysis of over 2.5 million vaccinees. Estimated myocarditis incidence of 2.13 cases per 100,000 persons overall, and 10.69 cases per 100,000 among men aged 16–29.
- The Lancet (Wong et al., 2022): US cohort study across four claims databases covering over 15 million people. Men aged 18–25 after a second mRNA dose: 1.71–2.17 cases per 100,000 person-days, depending on vaccine brand.
- The Lancet Respiratory Medicine (2022) systematic review and meta-analysis: 18.2 myopericarditis cases per million COVID-19 vaccine doses; young men disproportionately affected.
- The Lancet Child & Adolescent Health (Kracalik et al., 2022): CDC follow-up surveillance of adolescent and young-adult cases; 82 percent male, median age 21.
- eClinicalMedicine (MACiV multicenter study, 2024): Longitudinal data showing late gadolinium enhancement — a marker of myocardial scarring — in 82 percent of vaccine-associated myocarditis patients, with scarring persisting during convalescence.
- Science Translational Medicine (Stanford Medicine, December 2025): Mechanistic study identifying specific immune pathways by which mRNA vaccines can cause myocarditis.
- European Society of Cardiology Clinical Consensus Document (2022): Comprehensive clinical overview of vaccine-related myocarditis incidence, presentation, diagnosis, and management.

The same literature is clear on the comparative risk. SARS-CoV-2 infection itself carries a myocarditis risk approximately seven to ten times higher than vaccination. This context matters for honest framing: the vaccine-related injury risk is real, documented, and concentrated in specific demographic groups, but it is also lower than the risk from the disease the vaccine was designed to mitigate. This report does not argue against vaccination as a public health measure.

It argues for honest recognition of the injured population that the vaccination campaign produced, and for appropriate provincial administration of that recognition.

The structural conflict of interest

Between 2021 and 2022, the Government of Alberta implemented a series of measures tied to COVID-19 vaccination status. These included the Restrictions Exemption Program (which conditioned access to non-essential businesses on proof of vaccination), public sector employment mandates (which required vaccination or regular testing for specific categories of workers), and school and workplace policy frameworks that differentially treated vaccinated and unvaccinated individuals. These measures, regardless of their public health rationale, represented provincial government action that influenced, and in some cases effectively required, individual vaccination decisions.

A subset of those vaccinated individuals — nationally, at least 252 confirmed through VISP approval, with a larger population of claimants in review — experienced injuries meeting the federal standard for “serious and permanent.” An unknown proportion of that population resides in Alberta. Several of those individuals are, or will become, applicants to Alberta’s disability benefit system.

The structural situation is this: the provincial government that administered the vaccine mandate is the same provincial government that administers AISH, the Medical Review Panel that adjudicates AISH eligibility, and the Alberta Disability Assistance Program (ADAP) that will begin on July 1, 2026. The province has direct financial and political interest in the scale of the vaccine-injured population that is recognized within its disability benefit system. The province’s administrative approach to that population is not required to acknowledge the federal VISP/VIAP determination. Under Alberta’s disability benefit architecture, the federal recognition that an individual is seriously and permanently injured by a vaccine does not automatically translate to provincial recognition of that individual as eligible for AISH — or, after July 1, 2026, as eligible for continued AISH under the Medical Review Panel reassessment process.

If the Medical Review Panel, under the new ADAP framework, reclassifies a VISP-approved individual as employment-ready — under a standard the panel itself develops and applies — that individual will receive the reduced ADAP benefit, lose the earnings cap protection previously available under AISH, and be denied appeal rights on the reclassification decision. The federal determination remains in place. The federal compensation, if awarded, continues. But the provincial monthly benefit, the provincial health benefits, and the provincial administrative framework all operate on a parallel track that does not integrate the federal injury determination.

That is a structural conflict of interest. The government that created the circumstances of the injury is the government administering the benefit system used by the injured. There is no independent provincial body to adjudicate the injured population’s access to supports. The Medical Review Panel is appointed by the same government that imposed the mandate. The

benefit level is set by the same government. The appeal structure — or, under ADAP, the absence of one — is determined by the same government.

The administrative filter, applied

As documented in Section 16, the placation architecture operates through administrative instruments that appear responsive while producing the opposite outcome. For the vaccine-injured Albertan applying to AISH or ADAP, the filter manifests as follows:

- The injury may be coded medically under a category that does not explicitly attribute causation to vaccination (e.g., “idiopathic neurological condition,” “post-viral syndrome,” “autoimmune condition of unknown origin”). These codes can be accurate within the limits of the diagnostic framework, but they function, administratively, to break the causal chain between the provincial mandate and the federal injury determination.
- The Medical Review Panel under ADAP can, in principle, conclude that the individual's condition does not meet the AISH severity standard — even where the federal VISP review has determined that the injury is serious and permanent. The two determinations use different adjudicative frameworks and are not required to align.
- A denial by the Medical Review Panel under ADAP, once finalized, cannot be appealed. The individual remains on ADAP at the reduced benefit rate, without recourse.
- The individual's VISP compensation, if any, does not offset the provincial benefit reduction. VISP is one-time or ongoing based on its own criteria; ADAP is monthly and ongoing based on provincial criteria. The two do not integrate.

The result is a population of disabled people whose federal government has formally recognized their injury and its cause, and whose provincial government has the structural capacity to administratively minimize both the recognition and the supports attached to it. That population is small relative to the total AISH and ADAP caseload. It is not zero. And its administrative treatment is a visible case of the broader pattern this section has documented across four federal disability recognition mechanisms.

The pattern unified

Placing the four instances side by side produces a consistent structure:

Disability Tax Credit: federal mechanism for establishing disability recognition. Approximately 24 percent of eligible Canadians successfully enroll, constrained by application complexity and medical form cost. Alberta administration exacerbates rather than compensates for the barriers.

Registered Disability Savings Plan: federal mechanism for lifetime financial security. Federal grants and bonds allocated specifically for permanent disability. Fully exempt under Alberta AISH and ADAP as both asset and income. This is the exception in the pattern — and as an exception, it clarifies that the provincial position on other federal disability instruments is

selective rather than principled. Alberta exempts federal mechanisms that do not directly displace its own monthly payment role; it claws back federal mechanisms that would otherwise visibly supplement provincial payments.

Canada Disability Benefit: federal supplement explicitly designed to reduce disability poverty. Alberta is the only province or territory that claws it back. The \$200 per month federal dollar is withdrawn dollar-for-dollar from provincial AISH. The \$49 million provincial AISH reduction in Budget 2025 numerically approximates the provincial savings generated by the clawback.

Vaccine Injury Support Program: federal recognition of vaccine-attributed serious and permanent injury, with compensation. Approved claimants are federally certified as disabled. Alberta's disability benefit system is not required to acknowledge the federal determination and, under ADAP's Medical Review Panel framework, may reclassify the injured as employment-ready without right of appeal. The same government that mandated the vaccination administers the benefit system the injured access.

The pattern is not incidental. It is consistent across four distinct federal programs spanning tax policy, savings vehicles, income supplementation, and injury compensation. The provincial response, in each case, is structured in a way that reduces, offsets, or filters the federal recognition.

The unifying logic is fiscal. Alberta has received disproportionate revenue from resource extraction for decades. The Heritage Fund, as documented in the Sorochan–Knibbs collaborative synthesis elsewhere in this report series, held \$25.7 billion in assets as of 2024, with \$5.6 billion in deposits between 2022 and 2025. The province is not, by any defensible accounting, financially unable to supplement federal disability supports. It is, as a matter of policy, unwilling to do so.

The question is not whether Alberta can afford to support disabled Albertans. It is whether the provincial government regards that support as a priority relative to tax cuts, resource subsidies, and general fund surplus.

What this means for the July 1 transition

On July 1, 2026, approximately 79,290 Albertans will be transitioned from AISH to ADAP. A subset of them are Indigenous Albertans whose disability profile is shaped by the historical and environmental contexts documented in Sections 1 through 15 of this report. A subset of them are vaccine-injured Albertans whose disability has been federally recognized through VISIP or is still in federal review. Many are in both categories. All of them are about to be administered under a provincial framework that has, across four documented instances, structured its response to federal disability recognition as offset rather than amplification.

The ADAP transition is not a neutral administrative modernization. It is the most recent expression of a consistent provincial policy direction. The \$200 per month reduction, the stripped appeal rights, the Medical Review Panel architecture, the \$350 earnings cap replacing the previous \$1,072 threshold, and the absence of structural accommodation for federally-recognized injury categories are all features of the same design.

Federal disability recognition is not self-executing at the provincial level. Provincial administration is where the recognition either translates into lived support or is neutralized by the structure. In Alberta, across four instances and counting, the translation has been neutralization.

The remedy is not exclusively federal. It is the active and sustained reining in of a provincial government that has, on this file and others, decoupled its policy direction from the values, obligations, and commitments of the population it administers. That reining in is the work of this report series, the municipal coalitions, the Indigenous-led research institutions, the federal officials who can publicly acknowledge the provincial disconnect, and the disabled Albertans themselves, whose lives are the subject matter the policy claims to serve.

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